



Ultrasound in Pregnancy Procedure

1. Scope

Site	Operational Area	Applicable to
Armada Health Service	Obstetrics and Gynaecology	Medical and Nursing

2. Purpose

This procedure has been developed for the use of ultrasound consistent with evidence based best practice in obstetrics.

3. Procedure

3.1 First trimester

- All patients should be offered a first trimester ultrasound to confirm viability, gestational age, the number of fetuses and to exclude major abnormalities detectable at this time.
- Screening for trisomy 21 should be discussed with nuchal translucency (NT) assessment or the combined serum / NT offered.
- If ultrasound dates differ from those suggested by the LMP by > 7 days, then the ultrasound dates should be used.
- Cervical length assessment in woman with high risk factors for preterm delivery otherwise cervical assessment is required to be done in all women at their anatomy scan.

3.2 Second Trimester

- Fetal anatomy scans to be performed between 19-21 weeks period of gestation (POG). Cervical length assessment is to be done in all women at their anatomy scan.
- Cervical length is generally measured by TAS and should be $\geq 35\text{mm}$, if TAS shows cervical length $\leq 35\text{mm}$, then TVS is to be performed after informed verbal consent and explanation to the woman
- Explain the reason for performing this scan.
- Explain that ultrasound cannot detect all abnormalities and that it cannot detect developmental problems.
- If this is the first ultrasound that a patient has in the pregnancy the due date may need to be adjusted using head circumference and femur length measurements.
- Dates should not be changed at the second trimester ultrasound if already adjusted at first trimester scan
- There is no indication for umbilical artery Doppler's at this stage of the pregnancy unless there is severe IUGR or reduced amniotic fluid.

3.3 Third Trimester Scan and follow up scans

- Dichorionic twin pregnancy every 2-3 weeks from 26 weeks of gestation Monochorionic twin pregnancy every 2 -3 weeks from 16 weeks of gestation
- Low lying placenta rescans at 34-36 weeks
- Serial growth scans every 2-3 weeks in SGA, IUGR pregnancies and high-risk pregnancies
- High risk pregnancies including:
 - Maternal gestational Diabetes mellitus, scan at 32 and 36 weeks if well controlled
 - Hypertensive disorders in pregnancy
 - Obstetric cholestasis
 - Previous small for gestational age (SFG) baby
 - Antepartum haemorrhage (APH)
 - Previous FDIU
 - Maternal thrombophilia
 - Other medical disorders
 - Low PAPP-A, marginal cord and velamentous Cord insertion etc.
 - IVF pregnancies, advanced maternal age and CaLD women
- Weekly assessment of Maximum Vertical Pocket (MVP) and Uterine Artery Doppler in IUGR babies and in High-risk pregnancies where necessary
- Perform Middle Cerebral Artery Doppler (MCA) studies in suspected viral infections, severe IUGR, suspected fetal anaemia.
- Suspected large for gestation age babies, polyhydramnios and oligohydramnios.
- Fetal weight estimation to plan delivery for VBAC, if previous LSCS for macrosomic baby.
- Before ECV to confirm Breech presentation, placental position, amniotic fluid volume and fetal anomalies
- Formal ultrasound in motor vehicle accident woman to rule out abruption and assessment of fetal wellbeing.
- Third trimester scan for high BMI ladies with BMI ≥ 35
- Foetal wellbeing scan at 41 weeks in all pregnancies
- All CaLD ladies to have foetal biometry and wellbeing scan at 38 weeks
- Advanced maternal age ≥ 40 years to have third trimester scan at 38 weeks

3.4 Post Natal Scan

- Any woman with suspicion of retained products of conception
- Any woman with persistent bleeding post delivery
- Any concerns regarding woman's well-being post lower segment caesarean section (LSCS) or spontaneous vaginal birth (SVB).

When escalating care regarding a concern with the USS. The reporting midwife MUST co-sign with the medical officer the acknowledgement of results reviewed and/or any follow up care planned.

4. Legislative context

The following documents are required to give effect to this procedure:

- Health Department of Western Australia (HDWA) [Consent to Treatment Policy MP 0175/22](#)
- HDWA [Clinical Handover MP 0095](#)
- East Metropolitan Health Service (EMHS) [Policy Patient Identification and Procedure Matching EMHS: 21](#)

5. Supporting information

The following documents support the implementation of this procedure:

- [Antenatal Clinical Guideline](#)
- King Edward Memorial Hospital [Multiple Pregnancy Guideline](#)

6. Compliance, monitoring and evaluation

Compliance with this policy will be monitored via:

- Monitoring of reported clinical incident data via Datix CIMS.

Department specific findings are to be reported at the obstetrics safety and quality departmental meeting for review and identification of any opportunities for improvement.

7. Relevant standards

National Safety and Quality Health Service (NSQHS) Standards (second edition):

- Standard 2 – Action 2.4 Healthcare rights and informed consent
- Standard 5 – Action 5.11 Clinical assessment
- Standard 6 – Action 6.5 Correct identification and procedure matching
- Standard 6 – Action 6.7 Clinical handover
- Standard 6 – Action 6.11 Documentation of information

8. References

1. Australian Health Ministers' Advisory Council 2012, Clinical Practice Guidelines: Antenatal Care – Module 1. Australian Government Department of Health and Ageing, Canberra.
2. NICE clinical guideline 62. Antenatal care Routine care for the healthy pregnant woman Developed by the National Collaborating Centre for Women's and Children's Health Issue date: March 2008, Updated 2017
3. Monochorionic Twin Pregnancy, Management (Green-top51)
4. [Nice CG107 Hypertension in Pregnancy Diagnosis and Management](#)
5. [Management of Breech Presentation \(Green top guideline No. 20b\)](#)

Policy owner (relating to these procedures)

Enquiries relating to this procedure may be directed to:

Title: Head of Department
 Department: Obstetrics and Gynaecology
 Email: sangeeta.mallabhat@health.wa.gov.au

9. Review

This procedure will be reviewed and evaluated as required to ensure relevance and currency. At a minimum it will be reviewed within one (1) year after first issue and at least every three (3) years thereafter.

Version	Effective from	Effective to	Amendment(s)
Version 4	17/01/2024	17/01/2027	Scheduled review

10. Authorisation

This procedure has been authorised and issued by the Director Clinical Services.

Authorised by	Dr Tania Strickland – A/Director Clinical Services
Authorisation date	16/01/2024
Published date	17/01/2024